

Mantle Cell Lymphoma

Unified clinical guideline — v2.0 specialist guideline

MHA-MCL-2026-v2.0

Evidence and access cut-off: 28 July 2026

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Independent clinical review and pharmacy verification complete. Use with the current SmPC, local SACT protocol and jurisdiction-specific commissioning criteria.

Control status

This published release was generated from the controlled source and approved after independent clinical review, pharmacy verification and exact-hash owner authorisation.

1. Scope and use

This specialist specialist guideline covers adult mantle cell lymphoma from diagnosis through first-line and relapsed or refractory treatment. It separates clinical evidence, marketing authorisation, HTA recommendation and operational NHS access.

It is not a prescribing instruction, commissioning decision or substitute for the current SmPC, live national criteria, local SACT governance, MDT review and patient-specific judgement.

2. Diagnosis and minimum dataset

Confirm MCL through integrated morphology, immunophenotype and demonstration of cyclin D1 overexpression or an appropriate CCND-family rearrangement. Record nodal versus non-nodal presentation and obtain expert review when morphology, immunophenotype or genetics are discordant.

For uncommon cyclin D1-negative disease, require expert haematopathology review and additional molecular confirmation before assigning the diagnosis.

- Record morphology, including blastoid or pleomorphic features.
- Record Ki-67 using an agreed reproducible method.
- Assess TP53 status before treatment planning when technically feasible.
- Document a clinical risk score and the limitations of applying newer molecular models outside their validation setting.

3. Staging and risk

Baseline assessment should define disease distribution, tempo, symptoms, cytopenias, organ compromise, performance status, comorbidity and treatment fitness. TP53 mutation is a major adverse prognostic feature under intensive chemoimmunotherapy; it is prognostic rather than proof that one specific alternative strategy is superior.

Use molecular and pathological risk to support trial referral, treatment planning and consent. Do not claim that a novel regimen has neutralised high-risk biology unless comparative evidence demonstrates this.

4. Observation where treatment is not required

Selected asymptomatic patients with low-volume, indolent or non-nodal disease may be observed after diagnostic confidence and MDT review. Review clinically and with laboratory assessment every 3–6 months; use imaging when clinically required. Record the reason for observation and objective triggers for reassessment. Rising lymphocyte count or imaging change alone should be interpreted in its clinical context.

5. First-line pathway for younger or treatment-fit adults

Select treatment according to biological risk, treatment requirement, physiological fitness and suitability for dose-intensive treatment or ASCT—not chronological age alone.

Current conventional pathway when first-line covalent BTK inhibition is not routinely commissioned

Offer rituximab plus high-dose cytarabine-containing induction followed by ASCT consolidation and 3 years of rituximab maintenance. This is a bundled strategy: the MCL Younger trial does not isolate the independent effect of ASCT. Use the current network SACT/transplant protocol for the exact induction, conditioning, doses, administration and modifications.

After ASCT, the LYMA schedule was rituximab 375 mg/m² every 2 months for 3 years. It improved 4-year EFS, PFS and OS versus observation; prolonged follow-up retained the PFS benefit without a statistically significant OS difference. Confirm the current rituximab SmPC and local protocol before prescribing.

TRIANGLE-derived pathway

The mature TRIANGLE analysis applies to adults aged 18–65 who matched that protocol. Adding ASCT to the studied ibrutinib-containing regimen did not improve failure-free survival, while ibrutinib-containing arms improved reported overall survival versus control and increased infection burden.

The exact ibrutinib regimen has a GB marketing authorisation, but NICE GID-TA11802/ID6596 remains unfinished and its June 2026 recommendation is draft and negative. Do not present this pathway as routinely commissioned NHS treatment. If it is used through an authorised funding route, follow the exact SmPC regimen and do not add routine ASCT solely because the patient was initially transplant-eligible.

Rituximab maintenance within a TRIANGLE-derived pathway is recommended by the EHA–EU guideline, but the supporting TRIANGLE maintenance analysis was non-randomised and recorded more serious infection. It must not be represented as a risk-free default.

6. First-line pathway for older or transplant-ineligible adults

Treatment selection should integrate disease tempo, TP53 status, frailty, renal function, infection risk, cardiac and bleeding risk, drug interactions, treatment duration, patient preference and confirmed access.

Clinically actionable options

- Bendamustine–rituximab: offer as a standard first-line immunochemotherapy option when clinically appropriate, followed in responders by rituximab maintenance for at least 2 years. The BR comparison in StiL was a mixed indolent/MCL population, and maintenance-after-BR evidence includes observational data; retain those limits. Use the current local/network SACT protocol for exact doses and modifications.
- VR-CAP: consider as an alternative for previously untreated adults for whom HSCT is unsuitable. It improved PFS versus R-CHOP but caused more neutropenia and thrombocytopenia. NICE TA370 recommends it within its marketing authorisation. Rituximab maintenance for at least 2 years is an EHA–EU guideline recommendation and was not tested in the original VR-CAP trial.

Trial evidence that does not create routine NHS England access

- ENRICH: ibrutinib–rituximab improved PFS over the pooled immunochemotherapy control but did not establish superiority over BR specifically. No first-line NICE or national CDF route was identified.
- ECHO: acalabrutinib plus BR improved PFS without a demonstrated OS advantage. It is licensed, but NICE GID-TA11091/ID6155 remains unfinished and the February 2026 recommendation is draft and negative.
- SHINE: ibrutinib plus BR improved PFS without an OS advantage and increased grade 3–4 toxicity. No first-line NICE or national CDF route was identified.

7. TP53-mutated and other high-risk MCL

TP53-mutated disease should prompt early specialist discussion, explicit communication of uncertainty and consideration of an appropriate clinical trial. BOVEN and other response-adapted novel combinations show activity and feasibility, but available single-arm studies do not prove comparative superiority or elimination of TP53-associated risk.

MRD may refine prognosis and has been used to guide treatment cessation in selected studies. It is not a universal surrogate and does not justify omission of effective maintenance in routine practice.

8. Relapsed or refractory pathway

At each relapse, record prior regimens and classes, depth and duration of response, reason for stopping, current disease tempo, performance status, organ function, infection history and access constraints. Distinguish covalent-BTKi intolerance from progression.

- After exactly one previous line in England, ibrutinib and zanubrutinib have separate positive NICE recommendations subject to their exact criteria.

- At covalent-BTKi progression, assess cellular-therapy eligibility and refer early where appropriate. Manufacturing time and bridging risk must be considered.
- Pirtobrutinib has single-arm activity after covalent BTKi exposure but no demonstrated national MCL commissioning route at the access cut-off.
- At 67.8 months median follow-up in ZUMA-2 cohort 1, median duration of response with brexucabtagene autoleucel was 36.5 months. In England it remains a CDF managed-access route under TA677, subject to live national criteria.
- Lisocabtagene maraleucel has phase I activity after BTKi exposure but no demonstrated routine MCL route in England at the access cut-off.
- Glofitamab has fixed-duration phase I/II activity after prior BTKi but no current MCL marketing authorisation or routine commissioning route.
- Do not use unadjusted cross-trial response rates to rank pirtobrutinib, cellular therapy, bispecific antibodies or investigational BCL2 inhibition.

9. Response assessment, MRD and maintenance

Use a pre-specified response-assessment plan appropriate to the regimen and disease sites. Record whether MRD is being used for prognosis, trial-defined response adaptation or research. Do not use MRD negativity alone to omit maintenance outside a validated protocol.

Maintenance decisions must retain the exact induction population, evidence design, duration and infection burden. Evidence from one induction platform must not be silently transferred to another.

10. Supportive care and treatment safety

Before treatment, complete regimen-specific infection, vaccination, viral screening, drug-interaction, bleeding, cardiac, renal and tumour-lysis assessment. Use current local and national supportive-care policies for antimicrobial prophylaxis, immunoglobulin replacement, growth-factor support and vaccination.

The MCL efficacy evidence records clinically important infection, cytopenia and treatment-related mortality signals but does not establish one universal prophylaxis schedule. CAR-T and bispecific pathways require their dedicated supportive-care and toxicity-management protocols.

11. Follow-up and MDT documentation

After first-line systemic treatment, continue long-term follow-up with history, examination and laboratory assessment every 3–6 months initially. Document disease status, treatment exposure, response, toxicity, infection, patient priorities and the planned trigger for re-imaging or treatment. Routine surveillance imaging should be clinically directed. Suspected transformation or an unexpectedly aggressive relapse requires prompt re-staging and tissue confirmation where feasible.

Minimum audit dataset

- Record completion or documented infeasibility of morphology, Ki-67 and TP53 assessment before treatment planning.
- Record the clinical-evidence basis and the separately confirmed access route for each systemic treatment decision.
- At relapse, record whether a covalent BTK inhibitor stopped for intolerance or progression.
- For post-BTKi progression, record cellular-therapy eligibility assessment, referral decision and reason when referral is not made.
- Record planned follow-up interval and the clinical indication for surveillance imaging.

12. Regulatory and access boundary

The publication model uses an England access framework with separate devolved-nation notes. GB marketing authorisation, NICE recommendations, NHS England operational funding, SMC/NHSScotland, AWMSG/NHS Wales and HSCNI implementation are distinct determinations.

Absence of a national route does not prove that treatment is unavailable through a trial, early-access programme or individual decision. Those routes must be described as non-routine and confirmed before treatment.

13. Working regulatory and access matrix

Marketing authorisation, HTA recommendation, NHS commissioning and trial or exceptional access are separate determinations. All entries are time-sensitive and require live recheck before candidate freeze.

Conventional cytarabine-containing induction, ASCT and rituximab maintenance

Population	Fit adult suitable for dose-intensive treatment and ASCT when a first-line covalent BTK-inhibitor pathway is not routinely commissioned
Clinical evidence	EHA–EU guideline-supported conventional pathway; MCL Younger supports a bundled cytarabine-containing induction/conditioning/ASCT strategy and LYMA supports 3 years of post-ASCT rituximab maintenance
Marketing authorisation	No single combination marketing-authorisation statement was verified; each component, formulation and use must follow its current SmPC and the treating network protocol.
NICE / HTA	No single regimen-specific NICE technology appraisal was identified; absence of a TA is not proof of non-funding for generic chemotherapy or transplant services.
NHS England access	Confirm the current haemato-oncology network SACT and transplant pathway; do not infer access from the CDF list.
Devolved-nation notes	No single devolved-nation national route was audited for this complete conventional pathway; follow nation and network policy.
Regimen	Rituximab plus high-dose cytarabine-containing induction followed by ASCT consolidation and 3 years of rituximab maintenance.
Dose, schedule and duration	Evidence schedule, not a prescribing protocol: TRIANGLE primary report described six alternating 21-day R-CHOP/R-DHAP or R-DHAOx cycles. R-CHOP used rituximab 375 mg/m ² day 0/1, cyclophosphamide 750 mg/m ² day 1, doxorubicin 50 mg/m ² day 1, vincristine 1.4 mg/m ² day 1 and prednisone 100 mg days 1–5. R-DHAP/R-DHAOx used rituximab 375 mg/m ² day 0/1, dexamethasone 40 mg days 1–4, cytarabine 2 × 2 g/m ² over 3 hours 12-hourly on day 2 and cisplatin 100 mg/m ² over 24 hours day 1 or oxaliplatin 130 mg/m ² day 1. LYMA maintenance was rituximab 375 mg/m ² every 2 months for 3 years. Use the current network protocol for prescribing.
Administration/monitoring boundary	Component SmPC, stem-cell mobilisation, conditioning, organ-function thresholds, infection prophylaxis, fertility, supportive care and dose modification were not fully extracted into the pharmacy package. The current transplant/SACT protocol and human pharmacy verification are mandatory.
Pharmacy evidence status	REPORT_ONLY_TRIAL_AND_GUIDELINE_SCHEDULE_NOT_PHARMACY_VERIFIED
Provisional public wording	Guideline-supported conventional first-line pathway for a fit adult when first-line covalent BTK inhibition is not routinely commissioned, subject to current network SACT/transplant governance.

Official sources: <https://pmc.ncbi.nlm.nih.gov/articles/PMC12541557/> · <https://pubmed.ncbi.nlm.nih.gov/36469833/> · <https://pubmed.ncbi.nlm.nih.gov/28953447/>

Bendamustine-rituximab first line

Population	Adult unsuitable for dose-intensive treatment when BR is clinically appropriate
Clinical evidence	EHA–EU guideline-supported standard option; StiL provides pooled indolent/MCL randomised evidence and maintenance-after-BR evidence includes observational cohorts
Marketing authorisation	No single BR combination marketing-authorisation statement was verified in this work package; current component SmPCs and local protocol must be checked.
NICE / HTA	No regimen-specific first-line MCL NICE technology appraisal was identified; this does not establish absence of local or baseline access.
NHS England access	Use only through the current local/network SACT protocol.
Devolved-nation notes	Nation-specific and local implementation were not audited for this generic combination pathway.
Regimen	Bendamustine plus rituximab, followed in responders by rituximab maintenance for at least 2 years.
Dose, schedule and duration	Trial schedule, not a prescribing protocol: StiL used bendamustine 90 mg/m ² on days 1 and 2 of a 4-week cycle for up to six cycles plus rituximab 375 mg/m ² on day 1. The exact maintenance dose and interval were not extracted from the available verified recommendation; use the current local protocol.
Administration/monitoring boundary	The StiL result was pooled across indolent lymphomas and MCL and has a published erratum whose substantive correction was not retrieved. Component SmPCs, organ-function adjustments, infection prophylaxis and local SACT monitoring remain controlling; human pharmacy verification is pending.
Pharmacy evidence status	REPORT_ONLY_TRIAL_SCHEDULE_NOT_PHARMACY_VERIFIED
Provisional public wording	Guideline-supported standard first-line immunochemotherapy option for an adult unsuitable for dose-intensive treatment, with maintenance in responders, subject to current local SACT governance.

Official sources: <https://pmc.ncbi.nlm.nih.gov/articles/PMC12541557/> · <https://pubmed.ncbi.nlm.nih.gov/23433739/> · <https://pubmed.ncbi.nlm.nih.gov/35763708/>

VR-CAP / bortezomib combination

Population	Previously untreated adults for whom haematopoietic stem-cell transplantation is unsuitable
Clinical evidence	Clinical efficacy was not separately extracted in this targeted update; licence and NICE access are established for the defined population
Marketing authorisation	GB-authorized with rituximab, cyclophosphamide, doxorubicin and prednisone for untreated MCL when HSCT is unsuitable.
NICE / HTA	TA370 recommends use within the marketing authorisation.
NHS England access	Routine NICE-mandated access within TA370 scope; do not generalise to transplant-suitable patients.
Devolved-nation notes	SMC1075/15 accepted use in Scotland; Welsh implementation follows TA370; an HSCNI TA370 record was located.
Regimen	Bortezomib with rituximab, cyclophosphamide, doxorubicin and prednisone (VcR-CAP / VR-CAP)
Dose, schedule and duration	VELCADE 3.5 mg powder for solution for injection is administered via intravenous or subcutaneous injection at the recommended dose of 1.3 mg/m ² body surface area twice weekly for two weeks on days 1, 4, 8, and 11, followed by a 10-day rest period on days 12-21. This 3-week period is considered a treatment cycle. Six VELCADE cycles are recommended, although for patients with a response first documented at cycle 6, two additional VELCADE cycles may be given. At least 72 hours should elapse between consecutive doses of VELCADE. The following medicinal products are administered on day 1 of each VELCADE 3 week treatment cycle as intravenous infusions: rituximab at 375 mg/m ² , cyclophosphamide at 750 mg/m ² and doxorubicin at 50 mg/m ² . Prednisone is administered orally at 100 mg/m ² on days 1, 2, 3, 4 and 5 of each VELCADE treatment cycle.
Administration/monitoring boundary	VELCADE 3.5 mg powder for solution for injection is available for intravenous or subcutaneous administration. VELCADE should not be given by other routes. Intrathecal administration has resulted in death. VELCADE 3.5 mg reconstituted solution is administered as a 3-5 second bolus intravenous injection through a peripheral or central intravenous catheter followed by a flush with sodium chloride 9 mg/ml (0.9%) solution for injection. At least 72 hours should elapse between consecutive doses of VELCADE. Platelet counts should be $\geq 100,000$ cells/ μ L and the absolute neutrophils count (ANC) should be $\geq 1,500$ cells/ μ L Platelet counts should be $\geq 75,000$ cells/ μ L in patients with bone marrow infiltration or splenic sequestration Haemoglobin ≥ 8 g/dL Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT ONLY NOT PHARMACY VERIFIED – exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed and NICE-recommended in England for previously untreated adults for whom HSCT is unsuitable.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/3ebao6edb83ab9304ba520d874a7861e875a8237> · <https://www.nice.org.uk/guidance/ta370>

Ibrutinib monotherapy

Population	Relapsed or refractory MCL after exactly one previous line for the NICE-funded population
Clinical evidence	Covalent-BTKi treatment option; distinguish licence scope from NICE restriction
Marketing authorisation	GB licence covers adult relapsed or refractory MCL without the NICE one-line restriction.
NICE / HTA	TA502 recommends treatment only after one previous line, subject to the commercial arrangement.
NHS England access	Baseline funding recorded from 1 May 2018 under Blueteq IBR9 v1.1.
Devolved-nation notes	Scotland has SMC full-submission and resubmission records; Welsh advice is superseded by TA502; direct HSCNI implementation was not demonstrated in the audit.
Regimen	Ibrutinib monotherapy for relapsed or refractory MCL
Dose, schedule and duration	The recommended dose for the treatment of previously treated MCL is ibrutinib 560 mg once daily as a single agent. Treatment with IMBRUVICA as a single agent should continue until disease progression or no longer tolerated by the patient.
Administration/monitoring boundary	IMBRUVICA should be administered orally once daily with a glass of water approximately at the same time each day. The tablets should be swallowed whole with water and should not be broken or chewed. IMBRUVICA must not be taken with grapefruit juice or Seville oranges (see section 4.5). The dose of ibrutinib should be reduced to 280 mg once daily when used concomitantly with moderate CYP3A4 inhibitors. The dose of ibrutinib should be reduced to 140 mg once daily or withheld for up to 7 days when it is used concomitantly with strong CYP3A4 inhibitors. IMBRUVICA therapy should be withheld for any new onset or worsening grade 2 cardiac failure, grade 3 cardiac arrhythmias, grade ≥3 non-haematological toxicity, grade 3 or greater neutropenia with infection or fever, or grade 4 haematological toxicities. Monitor complete blood counts monthly. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED – exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Routinely funded in England after exactly one previous line when TA502 and live Blueteq criteria are met.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/9af68f2ddbce960e0e1f33927adff48693412b18> · <https://www.nice.org.uk/guidance/ta502>

Zanubrutinib monotherapy

Population	Relapsed or refractory MCL after exactly one previous line for the NICE-funded population
Clinical evidence	Covalent-BTKi option after prior therapy
Marketing authorisation	GB-authorised for adult MCL after at least one prior therapy.
NICE / HTA	TA1081 recommends an option after one line only, subject to the commercial arrangement and least-expensive-suitable-option wording.
NHS England access	Baseline funding recorded from 9 August 2025 under Blueteq ZAN6. Intolerance transfer from ibrutinib requires absence of progression.
Devolved-nation notes	SMC2819 applies in Scotland; NHS Wales has a TA1081 Blueteq route; direct HSCNI implementation was not demonstrated in the audit.
Regimen	Zanubrutinib monotherapy
Dose, schedule and duration	The recommended total daily dose of zanubrutinib is 320 mg. The daily dose may be taken either once daily (two 160 mg tablets) or divided into two doses of 160 mg twice daily (one 160 mg tablet). Treatment with BRUKINSA should be continued until disease progression or unacceptable toxicity.
Administration/monitoring boundary	BRUKINSA is for oral use. The film-coated tablets can be taken with or without food. Patients should be instructed to swallow the tablets whole with water, not to chew or crush the tablets. The tablet can be divided into two equal halves when advised by the healthcare provider. Patients with severe renal impairment (CrCl <30 mL/min) or on dialysis should be monitored for adverse reactions (see section 5.2). Monitor complete blood counts monthly during treatment (see section 4.2). Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED – exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Routinely funded in England after exactly one previous line when TA1081 and live Blueteq criteria are met.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/0660d09b59bb52f7204c3e4df1ff51083a38b21d> · <https://www.nice.org.uk/guidance/ta1081>

Brexucabtagene autoleucel

Population	Adult relapsed or refractory MCL after at least two systemic lines including a BTK inhibitor
Clinical evidence	Durable single-arm and real-world activity with material attrition, cytopenia, neurological and infection risks
Marketing authorisation	GB-authorized after at least two systemic lines including a BTK inhibitor.
NICE / HTA	TA677 remains a Cancer Drugs Fund managed-access recommendation while review continues.
NHS England access	NHS England list v1.405 retains KTE01a/KTE01b managed-access forms and national eligibility criteria. Do not describe this as unrestricted baseline commissioning.
Devolved-nation notes	SMC2351 is interim in Scotland; Welsh handling follows TA677 managed access; an HSCNI TA677 record was located.
Regimen	Brexucabtagene autoleucel
Dose, schedule and duration	Treatment consists of a single dose for infusion containing a dispersion for infusion of CAR-positive viable T cells in one infusion bag. The target dose is 2×10^6 CAR-positive viable T cells per kg of body weight (range: 1×10^6 – 2×10^6 cells/kg), with a maximum of 2×10^8 CAR-positive viable T cells for patients 100 kg and above. Tecartus is recommended to be infused 3 to 14 days after completion of the lymphodepleting chemotherapy for MCL patients. A lymphodepleting chemotherapy regimen consisting of cyclophosphamide 500 mg/m ² intravenously and fludarabine 30 mg/m ² intravenously must be administered prior to infusing Tecartus. The recommended days are on the 5th, 4th, and 3rd day before infusion of Tecartus.
Administration/monitoring boundary	Tecartus must be administered in a qualified treatment centre by a physician with experience in the treatment of haematological malignancies and trained for administration and management of patients treated with Tecartus. At least 1 dose of tocilizumab for use in the event of cytokine release syndrome (CRS) and emergency equipment must be available prior to infusion. Tecartus is intended for autologous use only (see section 4.4). Patients must be monitored daily for the first 7 days following infusion for signs and symptoms of potential CRS, neurologic events and other toxicities. Patients must remain within proximity of a qualified treatment centre for at least 4 weeks following infusion. Unresolved serious adverse reactions (especially pulmonary reactions, cardiac reactions, or hypotension) including from preceding chemotherapies. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED — exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Available in England through TA677 Cancer Drugs Fund managed access when all live national eligibility and panel criteria are met.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/cd6be42f02be6ec2bd92569340aedaf6f1179f16> · <https://www.nice.org.uk/guidance/ta677>

TRIANGLE ibrutinib regimen

Population	Previously untreated adults eligible for ASCT
Clinical evidence	Mature randomised phase III evidence supports the studied ibrutinib-containing non-ASCT strategy; applicability is protocol-specific.
Marketing authorisation	GB-authorized as ibrutinib with R-CHOP alternating with R-DHAP or R-DHAOx without ibrutinib, followed by ibrutinib monotherapy.
NICE / HTA	GID-TA11802/ID6596 remains in progress. June 2026 draft guidance recommends not using the regimen; this is not final guidance.
NHS England access	No final NICE entitlement or demonstrated national commissioning route at the cut-off.
Devolved-nation notes	No positive national route was demonstrated in Scotland, Wales or Northern Ireland.
Regimen	Ibrutinib with R-CHOP alternating with R-DHAP/R-DHAOx, followed by ibrutinib monotherapy
Dose, schedule and duration	The recommended dose for the treatment of previously untreated MCL is ibrutinib 560 mg once daily (see Table 1). Table transcription: cycles_1_3_5: IMBRUVICA in combination with R-CHOP; On days 1-19; cycles_2_4_6: R-DHAP; Without IMBRUVICA; part_i_footnote: 6 cycles; each cycle is 21 days.; part_ii: IMBRUVICA; Daily for 24 Months; r_dhap_footnote: Interchangeable with R-DHAOx (rituximab, dexamethasone, cytarabine, oxaliplatin). Boundary: See the Summary of Product Characteristics (SmPC) for dosing information for each medicinal product. No R-CHOP, R-DHAP or R-DHAOx component doses are supplied here because this ibrutinib SmPC explicitly redirects to each component SmPC.
Administration/monitoring boundary	Treatment should start after recovery of peripheral blood counts. Rituximab may be added as per national treatment guidelines. IMBRUVICA should be administered orally once daily with a glass of water approximately at the same time each day. The tablets should be swallowed whole with water and should not be broken or chewed. The SmPC MCL dose-modification and monitoring constraints apply to ibrutinib; combination-agent modifications require the respective live SmPCs and protocol verification. Uses the same checked SmPC modification/monitoring boundaries as ibrutinib monotherapy for relapsed or refractory MCL. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED — exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed but not routinely commissioned nationally in England at the cut-off; the current negative NICE recommendation is draft, not final.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/9af68f2ddbce960e0e1f33927adff48693412b18> · <https://www.nice.org.uk/guidance/indevelopment/gid-ta11802>

Acalabrutinib plus bendamustine–rituximab

Population	Previously untreated adults not eligible for ASCT
Clinical evidence	Randomised phase III progression-free survival benefit without demonstrated overall-survival advantage at reported follow-up
Marketing authorisation	GB-authorized in the defined first-line transplant-ineligible population.
NICE / HTA	GID-TA11091/ID6155 remains in progress; February 2026 draft guidance recommends not using it. The displayed expected-publication date is overdue.
NHS England access	No final recommendation or demonstrated national MCL Blueteq route.
Devolved-nation notes	SMC2929 is under consideration; no final positive Welsh or Northern Irish route was demonstrated.
Regimen	Acalabrutinib plus bendamustine and rituximab
Dose, schedule and duration	The recommended dose of Calquence in monotherapy or in combination with other medicinal products is 100 mg acalabrutinib twice daily (equivalent to a total daily dose of 200 mg). Calquence dose interval is approximately 12 hours. Calquence should be administered from Day 1 on Cycle 1 (each cycle is 28 days) continuously until disease progression or unacceptable toxicity. Bendamustine should be administered at 90 mg/m ² on Days 1 and 2 of each cycle for a total of 6 cycles. Rituximab should be administered at 375 mg/m ² on Day 1 each cycle for a total of 6 cycles. Patients achieving a response (partial response [PR] or complete response [CR]) after the first 6 cycles, may receive maintenance rituximab at 375 mg/m ² on Day 1 of every other cycle for a maximum of 12 additional doses, starting on Cycle 8 up to Cycle 30.
Administration/monitoring boundary	For the combination regimens, refer to the prescribing information of each of the medicinal products for their dosing information (for details of the combination regimens, see section 5.1). Calquence is for oral use. The tablets should be swallowed whole with water at approximately the same time each day, with or without food (see section 4.5). The tablets should not be chewed, crushed, dissolved or divided. Recommended dose modifications for Grade $\geq$ 3 adverse reactions in patients receiving Calquence in combination with bendamustine and rituximab are provided in Table 2. Refer to the prescribing information of each of the medicinal products used in combination with Calquence for additional information for management of toxicities. If these inhibitors will be used short-term (such as anti-infectives for up to seven days), interrupt Calquence. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED — exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed but without a demonstrated routine national route in England; current draft NICE advice is not final guidance.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/24b093c3ac16e4ad56f01258dbfa998647538a16> · <https://www.nice.org.uk/guidance/indevelopment/gid-ta11091>

Acalabrutinib monotherapy

Population	Adult relapsed or refractory MCL not previously treated with a BTK inhibitor
Clinical evidence	Licensed covalent-BTKi option requiring separate clinical appraisal
Marketing authorisation	GB-authorized for BTKi-naïve relapsed or refractory MCL.
NICE / HTA	GID-TA11470/ID6389 is awaiting development; publication TBC.
NHS England access	No demonstrated national MCL commissioning route.
Devolved-nation notes	No final positive national route was demonstrated in Scotland, Wales or Northern Ireland.
Regimen	Acalabrutinib monotherapy for relapsed or refractory MCL
Dose, schedule and duration	The recommended dose of Calquence in monotherapy or in combination with other medicinal products is 100 mg acalabrutinib twice daily (equivalent to a total daily dose of 200 mg). Calquence dose interval is approximately 12 hours. Treatment with Calquence in monotherapy or in combination with obinutuzumab should be continued until disease progression or unacceptable toxicity.
Administration/monitoring boundary	Calquence is for oral use. The tablets should be swallowed whole with water at approximately the same time each day, with or without food (see section 4.5). The tablets should not be chewed, crushed, dissolved or divided. Use the acalabrutinib-specific dose modification and monitoring constraints; no combination-agent rules apply to this monotherapy record. Uses the same checked SmPC modification/monitoring boundaries as acalabrutinib plus bendamustine and rituximab. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED – exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed for BTKi-naïve relapsed or refractory MCL but not demonstrated as routinely commissioned nationally in England at the cut-off.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/24b093c3ac16e4ad56f01258dbfa998647538a16> · <https://www.nice.org.uk/guidance/awaiting-development/gid-ta11470>

Pirtobrutinib monotherapy

Population	Adult relapsed or refractory MCL previously treated with a BTK inhibitor
Clinical evidence	Single-arm post-covalent-BTKi activity; no comparative sequencing hierarchy established
Marketing authorisation	GB-authorized after previous BTK-inhibitor treatment.
NICE / HTA	GID-TA10858/ID3975 returned to the NICE work programme on 14 July 2026 and remains in progress.
NHS England access	No demonstrated national MCL commissioning route; CLL access must not be transposed to MCL.
Devolved-nation notes	SMC2897 is a non-submission route; no positive national Welsh or Northern Irish MCL route was demonstrated.
Regimen	Pirtobrutinib monotherapy
Dose, schedule and duration	The recommended dose is 200 mg pirtobrutinib once daily (QD). Treatment should be continued until disease progression or unacceptable toxicity.
Administration/monitoring boundary	The tablet should be swallowed whole with a glass of water to ensure consistent performance (patients should not chew, crush, or split tablets before swallowing) and can be taken with or without food. Patients should take the dose at approximately the same time every day. Jaypirca dosing should be interrupted until recovery to Grade 1 or baseline when the patient experiences the following event: Grade 3 neutropenia with fever and/or infection Grade 4 neutropenia lasting ≥ 7 days Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED – exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed after prior BTKi exposure but not demonstrated as routinely commissioned nationally for MCL in England at the cut-off.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/1d71deb47f55d9284beab5786752e8368d37d284> · <https://www.nice.org.uk/guidance/indevelopment/gid-ta10858>

Lisocabtagene maraleucel

Population	Adult relapsed or refractory MCL after at least two systemic lines including a BTK inhibitor
Clinical evidence	Phase I activity; no head-to-head comparison with brexucel or pirtobrutinib
Marketing authorisation	GB-authorised after at least two systemic lines including a BTK inhibitor.
NICE / HTA	GID-TA11930 is awaiting development; the live page displays conflicting identifiers ID6695 and 12299.
NHS England access	No demonstrated national MCL commissioning route.
Devolved-nation notes	SMC2909 is awaiting a decision; no national Welsh or Northern Irish MCL route was demonstrated.
Regimen	Lisocabtagene maraleucel
Dose, schedule and duration	Breyanzi is intended for autologous use (see section 4.4). Treatment consists of a single dose for infusion containing a dispersion for infusion of CAR-positive viable T-cells in one or more vials. The target dose is 100×10^6 CAR-positive viable Tcells (consisting of a target 1:1 ratio of CD4+ and CD8+ cell components) within a range of $44\text{--}120 \times 10^6$ CAR-positive viable T-cells. See the accompanying release for infusion certificate (RfIC) for additional information pertaining to dose. Lymphodepleting chemotherapy consisting of cyclophosphamide 300 mg/m ² /day and fludarabine 30 mg/m ² /day, administered intravenously for three days. See the prescribing information for fludarabine and cyclophosphamide for information on dose adjustment in renal impairment. Breyanzi is to be administered 2 to 7 days after completion of lymphodepleting chemotherapy.
Administration/monitoring boundary	Breyanzi must be administered in a qualified treatment centre. At least 1 dose of tocilizumab for use in the event of cytokine release syndrome (CRS) and emergency equipment must be available per patient prior to infusion of Breyanzi. It is recommended that premedication with paracetamol and diphenhydramine (25–50 mg, intravenously or orally) or another H ₁ -antihistamine, be administered 30 to 60 minutes before the infusion of Breyanzi to reduce the possibility of an infusion reaction. Patients should be monitored 2–3 times during the first week following infusion, for signs and symptoms of potential CRS, neurologic events and other toxicities. Frequency of monitoring after the first week should be carried out at the physician's discretion and should be continued for at least 2 weeks after infusion. Patients should be instructed to remain within proximity of a qualified treatment centre for at least 2 weeks following infusion. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED — exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed for the defined post-BTKi population but not demonstrated as routinely commissioned nationally for MCL in England at the cut-off.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/ad49533cc5d81176e83f3082da1669b3a9e543ce> · <https://www.nice.org.uk/guidance/awaiting-development/gid-ta11930>

Lenalidomide monotherapy

Population	Adult relapsed or refractory MCL
Clinical evidence	Licensed non-routine option; combination with rituximab must not be conflated with monotherapy
Marketing authorisation	GB-authorised as monotherapy for adult relapsed or refractory MCL.
NICE / HTA	TA774 was terminated because no evidence submission was provided; this is no recommendation, not a negative clinical recommendation.
NHS England access	No national MCL Blueteq route demonstrated; NICE directs rational local decision-making.
Devolved-nation notes	SMC1211/16 was not recommended following non-submission; Wales did not endorse following non-submission; no Northern Irish national route was demonstrated.
Regimen	Lenalidomide monotherapy
Dose, schedule and duration	The recommended starting dose of lenalidomide is 25 mg orally once daily on days 1 to 21 of repeated 28-day cycles.
Administration/monitoring boundary	Revlimid capsules should be taken orally at about the same time on the scheduled days. The capsules should not be opened, broken or chewed. The capsules should be swallowed whole, preferably with water, either with or without food. Dose is modified based upon clinical and laboratory findings (see section 4.4). Dose adjustments, during treatment and restart of treatment, are recommended to manage Grade 3 or 4 thrombocytopenia, neutropenia, or other Grade 3 or 4 toxicity judged to be related to lenalidomide. All patients should receive TLS prophylaxis (allopurinol, rasburicase or equivalent as per institutional guidelines) and be well hydrated (orally) during the first week of the first cycle or for a longer period if clinically indicated. Full section 4.2/4.4 constraints remain controlling; use the live SmPC and local protocol.
Pharmacy evidence status	REPORT_ONLY_NOT_PHARMACY_VERIFIED — exact MHRA SmPC quotations validated; human pharmacy verification pending.
Provisional public wording	Licensed monotherapy without a positive national recommendation; any use requires an explicit locally governed access route.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/4eee1eaeac146ee6d44f54c544a0a61d13407294> · <https://www.nice.org.uk/guidance/ta774>

Ibrutinib plus venetoclax

Population	Relapsed or refractory MCL
Clinical evidence	Randomised progression-free survival evidence does not itself establish a licensed or commissioned route
Marketing authorisation	No current MCL marketing authorisation for the combination.
NICE / HTA	GID-TA10774/ID3879 was suspended on 20 January 2026 because an MHRA application was no longer being pursued.
NHS England access	No demonstrated national commissioning route.
Devolved-nation notes	No positive national MCL route was demonstrated in Scotland, Wales or Northern Ireland.
Regimen	No licensed MCL regimen established in the checked status matrix.
Dose, schedule and duration	Not applicable for MCL: no current GB MCL indication was recorded; do not transpose dosing from another disease.
Administration/monitoring boundary	Do not use another-indication SmPC or commissioning pathway as an MCL protocol.
Pharmacy evidence status	NOT_APPLICABLE_NO_CURRENT_GB_MCL_INDICATION
Provisional public wording	Not licensed or routinely commissioned nationally for MCL; normally limited to a trial or specifically approved exceptional route.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/6ff063d046996d11ab3bd16657a86a4f5430e485> · <https://www.nice.org.uk/guidance/indevelopment/gid-ta10774>

Glofitamab

Population	Relapsed or refractory MCL, including after BTKi in the phase I/II evidence
Clinical evidence	Fixed-duration phase I/II activity; non-randomised and not comparative sequencing evidence
Marketing authorisation	No current GB MCL indication.
NICE / HTA	GID-TA12447 is awaiting development for MCL.
NHS England access	No demonstrated national MCL commissioning route; DLBCL routes must not be extrapolated.
Devolved-nation notes	No positive national MCL route was demonstrated in Scotland, Wales or Northern Ireland.
Regimen	No licensed MCL regimen established in the checked status matrix.
Dose, schedule and duration	Not applicable for MCL: no current GB MCL indication was recorded; do not transpose dosing from another disease.
Administration/monitoring boundary	Do not use another-indication SmPC or commissioning pathway as an MCL protocol.
Pharmacy evidence status	NOT_APPLICABLE_NO_CURRENT_GB_MCL_INDICATION
Provisional public wording	Emerging evidence-based option without a current MCL licence or demonstrated routine national commissioning route.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/be3b230112f38c96d3dd8218c623e4acbc188a32> · <https://www.nice.org.uk/guidance/awaiting-development/gid-ta12447>

Epcoritamab

Population	Relapsed or refractory MCL
Clinical evidence	No qualifying PubMed-indexed primary MCL trial was established in the targeted evidence audit
Marketing authorisation	No current GB MCL indication.
NICE / HTA	No matching MCL NICE appraisal was found in the documented live search.
NHS England access	No demonstrated national MCL commissioning route; other-lymphoma routes must not be extrapolated.
Devolved-nation notes	No positive national MCL route was demonstrated in Scotland, Wales or Northern Ireland.
Regimen	No licensed MCL regimen established in the checked status matrix.
Dose, schedule and duration	Not applicable for MCL: no current GB MCL indication was recorded; do not transpose dosing from another disease.
Administration/monitoring boundary	Do not use another-indication SmPC or commissioning pathway as an MCL protocol.
Pharmacy evidence status	NOT_APPLICABLE_NO_CURRENT_GB_MCL_INDICATION
Provisional public wording	Not established as a routine MCL option; no current MCL licence, NICE appraisal or national commissioning route was demonstrated.

Official sources: <https://mhraproducts4853.blob.core.windows.net/docs/396fbcf79471ec18c6096563395a6213231113b4>

Evidence references

Targeted independently verified evidence update; not a formal systematic review. Scientific extraction is abstract-only except where the controlled evidence ledger states otherwise.

- S01:** Jerkeman M et al. EHA–EU MCL network guidelines for diagnosis and treatment of mantle cell lymphoma. HemaSphere. 2025. PMID 41132246; DOI 10.1002/hem3.70233 | Design/population: Clinical guideline | Verified extraction: Verified full text supports integrated diagnosis, morphology/Ki-67/TP53 assessment, observation of selected asymptomatic low-risk disease, re-biopsy at relapse and long-term follow-up | Integrity: V2-CONFIRMED; full text checked at PMCID PMC12541557; no correction/retraction found
- S02:** Jain P et al. High-risk MCL: recognition and treatment. Blood. 2025. PMID 39786418; DOI 10.1182/blood.2023022354 | Design/population: Review | Verified extraction: High-risk clinical, pathological and molecular features | Integrity: V2; secondary evidence
- S03:** Bastos-Boente M et al. Development and validation of a novel prognostic index for mantle cell lymphoma integrating TP53 mutations (MIPI53). Br J Haematol. 2026. PMID 42134850; DOI 10.1111/bjh.70535 | Design/population: Development/validation; N=143 plus external cohort | Verified extraction: 5-year PFS 83.1%/35.4%/12.0%; external c-index 0.732 | Integrity: V2; no issue found
- S04:** Eskelund CW et al. TP53 mutations identify younger mantle cell lymphoma patients who do not benefit from intensive chemoimmunotherapy. Blood. 2017. PMID 28819011; DOI 10.1182/blood-2017-04-779736 | Design/population: Molecular cohort; N=183 | Verified extraction: TP53 OS HR 6.2; median OS 1.8 vs 12.7 years | Integrity: V2; comment linked, no correction/retraction
- S05:** Moia R et al. Molecular biomarkers as key determinants of outcome in mantle cell lymphoma: results from the FIL V-RBAC trial. Blood Adv. 2026. PMID 42498280; DOI 10.1182/bloodadvances.2026020780 | Design/population: Biomarker analysis; N=132 | Verified extraction: Four independently associated molecular variables/model | Integrity: V2; newly indexed 24 July 2026
- S06:** Dreyling M et al. Addition of autologous stem-cell transplantation to an ibrutinib-containing first-line treatment... (TRIANGLE): 4.5-year follow-up... Lancet. 2026. PMID 42134356; DOI 10.1016/S0140-6736(26)00362-4 | Design/population: Randomised phase III; N=870 | Verified extraction: 4-year FFS 82%/81%/70%; no added ASCT benefit; both ibrutinib arms improved 4-year OS versus control; grade 3-5 infections 34%/26%/15% | Integrity: V2; no issue found
- S07:** Lewis DJ et al. Ibrutinib and rituximab versus immunochemotherapy... (ENRICH). Lancet. 2025. PMID 41052510; DOI 10.1016/S0140-6736(25)01432-1 | Design/population: Randomised phase II/III; N=397 | Verified extraction: PFS HR 0.69 | Integrity: V2; no issue found
- S08:** Wang M et al. Acalabrutinib Plus Bendamustine-Rituximab in Untreated Mantle Cell Lymphoma. J Clin Oncol. 2025. PMID 40311141; DOI 10.1200/JCO-25-00690 | Design/population: ECHO, randomised phase III; N=598 | Verified extraction: PFS 66.4 vs 49.6 months; OS NS | Integrity: V2; no issue found

9. **S09:** Kumar A et al. Zanubrutinib, obinutuzumab, and venetoclax for first-line treatment of mantle cell lymphoma with a TP53 mutation. *Blood*. 2025. PMID 39437708; DOI 10.1182/blood.2024025563 | Design/population: BOVen phase II; N=25 | Verified extraction: ORR 96%; CR 88%; 2-year PFS 72% | Integrity: V2; no issue found
10. **S10:** Visco C et al. Rituximab, bendamustine, and cytarabine followed by venetoclax... (FIL_V-RBAC). *Lancet Haematol*. 2025. PMID 40975105; DOI 10.1016/S2352-3026(25)00252-2 | Design/population: Single-arm phase II; N=140, high-risk n=54 | Verified extraction: High-risk 2-year PFS 60% | Integrity: V2; no issue found
11. **S11:** Ruan J et al. MRD-driven initial therapy of acalabrutinib and lenalidomide plus rituximab or obinutuzumab for mantle cell lymphoma. *Blood Adv*. 2026. PMID 41289154; DOI 10.1182/bloodadvances.2025017760 | Design/population: Phase II | Verified extraction: ALR ORR 100%; molecular CR 67% after 12 cycles | Integrity: V2; cohort denominators incomplete in abstract
12. **S12:** Jerkeman M et al. MRD-driven treatment with venetoclax-R2... MCL7 VALERIA trial. *Blood Adv*. 2024. PMID 38113470; DOI 10.1182/bloodadvances.2023011920 | Design/population: Phase Ib/II; N=59 | Verified extraction: ORR 63%; PFS 21 months; 28 stopped in molecular remission | Integrity: V2-CORRECTED
13. **S12C:** Erratum: Jerkeman M... VALERIA trial. *Blood Adv*. 2024. PMID 39392649; DOI 10.1182/bloodadvances.2024013955 | Design/population: Published erratum | Verified extraction: Correct lenalidomide phase II dose: 15 mg, not 20 mg | Integrity: V2-CONFIRMED CORRECTION
14. **S13:** Hoster E et al. Predictive Value of Minimal Residual Disease for Efficacy of Rituximab Maintenance... *J Clin Oncol*. 2024. PMID 37992261; DOI 10.1200/JCO.23.00899 | Design/population: Prospective planned trial analysis | Verified extraction: MRD-negative PFS HR 0.38 with maintenance | Integrity: V2; no issue found
15. **S14:** Wang ML et al. Pirtobrutinib in Covalent Bruton Tyrosine Kinase Inhibitor Pretreated Mantle-Cell Lymphoma. *J Clin Oncol*. 2023. PMID 37192437; DOI 10.1200/JCO.23.00562 | Design/population: BRUIN phase I/II; efficacy n=90 | Verified extraction: ORR 57.8%; CR 20%; DOR 21.6 months | Integrity: V2; no issue found
16. **S15:** Muñoz J et al. Five-year follow-up... ZUMA-2, Cohorts 1 and 2. *J Hematol Oncol*. 2026. PMID 42036693; DOI 10.1186/s13045-026-01797-4 | Design/population: Five-year single-arm follow-up; ZUMA-2 cohort 1 N=68 | Verified extraction: At median follow-up 67.8 months, median DOR was 36.5 months and median OS was 46.5 months | Integrity: V2; no issue found
17. **S16:** Wang M et al. Lisocabtagene Maraleucel in Relapsed/Refractory Mantle Cell Lymphoma... TRANSCEND NHL 001. *J Clin Oncol*. 2024. PMID 38072625; DOI 10.1200/JCO.23.02214 | Design/population: Phase I; 104 leukapheresed, 88 infused | Verified extraction: ORR 83.1%; CR 72.3%; PFS 15.3 months | Integrity: V2; no issue found
18. **S17:** Ahmed N et al. Real-world outcomes of brexucabtagene autoleucel... a CIBMTR analysis. *Blood Adv*. 2025. PMID 40706035; DOI 10.1182/bloodadvances.2024015014 | Design/population: Prospective registry; N=476 | Verified extraction: ORR 91%; CR 82%; 1-year PFS 63% | Integrity: V2; observational
19. **S18:** O'Reilly MA et al. Brexucabtagene autoleucel... in the United Kingdom: A real-world intention-to-treat analysis. *HemaSphere*. 2024. PMID 38873532; DOI 10.1002/hem3.87 | Design/population: UK ITT; 119 approved, 83 infused | Verified extraction: Infused ORR 87%; 24-month NRM 25%, mainly infection | Integrity: V2; observational
20. **S19:** Phillips TJ et al. Glofitamab in Relapsed/Refractory Mantle Cell Lymphoma: Results From a Phase I/II Study. *J Clin Oncol*. 2025. PMID 39365960; DOI 10.1200/JCO.23.02470 | Design/population: Phase I/II; evaluable n=60 | Verified extraction: ORR 85%; CR 78.3%; prior-BTKi ORR 74.2% | Integrity: V2; no issue found
21. **S20:** Eyre TA et al. Phase I/II Study of Sonrotoclax (BGB-11417) Monotherapy... *J Clin Oncol*. 2026. PMID 42385124; DOI 10.1200/JCO-26-00550 | Design/population: Phase I/II; efficacy n=103 | Verified extraction: Abstract reports ORR 52.4%; PFS 6.5 months | Integrity: V2-CORRECTED; correction content unresolved
22. **S20C:** Eyre TA et al. Erratum: Phase I/II Study of Sonrotoclax... *J Clin Oncol*. 2026. PMID 42447415; DOI 10.1200/JCO-26-01699 | Design/population: Published erratum | Verified extraction: Correction relationship established; corrected field not available in checked metadata | Integrity: V2-CONFIRMED CORRECTION
23. **S21:** Kluin-Nelemans HC et al. Treatment of Older Patients With Mantle Cell Lymphoma: Long-Term Follow-Up of the Randomized European MCL Elderly Trial. *J Clin Oncol*. 2020. PMID 31804876; DOI 10.1200/JCO.19.01294 | Design/population: Randomised phase III | Verified extraction: After R-CHOP, maintenance PFS 5.4 vs 1.9 years | Integrity: V2; no issue found
24. **S22:** Wang ML et al. Ibrutinib plus Bendamustine and Rituximab in Untreated Mantle-Cell Lymphoma. *N Engl J Med*. 2022. PMID 35657079; DOI 10.1056/NEJMoa2201817 | Design/population: SHINE randomised; N=523 | Verified extraction: PFS 80.6 vs 52.9 months; OS similar | Integrity: V2; comments linked, no correction/retraction
25. **S23:** Ladetto M et al. Rituximab Maintenance Added to Ibrutinib-Containing Therapy in Younger, Untreated Patients With Mantle Cell Lymphoma: Results From the TRIANGLE Trial. *J Clin Oncol*. 2026. PMID 42447409; DOI 10.1200/JCO-26-00705 | Design/population: Non-randomised secondary TRIANGLE analysis | Verified extraction: Higher 4-year PFS and grade 3–5 infections with maintenance | Integrity: V2; residual confounding
26. **S24:** Dreyling M et al. Ibrutinib combined with immunochemotherapy with or without autologous stem-cell transplantation... (TRIANGLE). *Lancet*. 2024. PMID 38705160; DOI 10.1016/S0140-6736(24)00184-3 | Design/population: Randomised open-label phase III; N=870; untreated stage II–IV MCL, age 18–65 years, suitable for ASCT | Verified extraction: At 31 months, 3-year FFS was 88% with ibrutinib plus ASCT, 86% with ibrutinib without ASCT and 72% with conventional ASCT; conventional ASCT superiority over ibrutinib without ASCT was not shown | Integrity: V2-CONFIRMED PubMed/Crossref; comment linked; no correction/retraction found
27. **S25:** Rummel MJ et al. Bendamustine plus rituximab versus CHOP plus rituximab as first-line treatment for patients with indolent and mantle-cell lymphomas. *Lancet*. 2013. PMID 23433739; DOI 10.1016/S0140-6736(12)61763-2 | Design/population: Randomised open-label phase III; mixed indolent lymphoma and MCL population | Verified extraction: Pooled trial results favoured BR over R-CHOP for PFS and several toxicities; the abstract does not provide an MCL-specific treatment effect | Integrity: V2-CORRECTED PubMed/Crossref; published erratum DOI 10.1016/S0140-6736(13)60801-6 identified but substantive correction not retrieved; keep pooled findings qualified

28. **S26:** Hermine O et al. High-Dose Cytarabine and Autologous Stem-Cell Transplantation in Mantle Cell Lymphoma: Long-Term Follow-Up of the Randomized MCL Younger Trial. *J Clin Oncol.* 2023. PMID 36469833; DOI 10.1200/JCO.22.01780 | Design/population: Randomised phase III long-term update; advanced MCL, age under 66 years | Verified extraction: The bundled R-CHOP/R-DHAP, cytarabine-containing conditioning and ASCT strategy improved median time to treatment failure to 8.4 versus 3.9 years compared with R-CHOP/ASCT | Integrity: V2-CONFIRMED PubMed/Crossref; no correction/retraction found
29. **S27:** Le Gouill S et al. Rituximab after Autologous Stem-Cell Transplantation in Mantle-Cell Lymphoma. *N Engl J Med.* 2017. PMID 28953447; DOI 10.1056/NEJMoa1701769 | Design/population: Randomised phase III; 240 patients randomised after ASCT | Verified extraction: Rituximab 375 mg/m² every 2 months for 3 years improved 4-year EFS, PFS and OS versus observation | Integrity: V2-CONFIRMED PubMed/Crossref; no correction/retraction found
30. **S28:** Sarkozy C et al. Long-Term Follow-Up of Rituximab Maintenance in Young Patients With Mantle-Cell Lymphoma Included in the LYMA Trial. *J Clin Oncol.* 2024. PMID 38109684; DOI 10.1200/JCO.23.01586 | Design/population: Long-term LYMA trial follow-up; median follow-up 7.5 years | Verified extraction: Seven-year PFS was 78.5% with rituximab maintenance versus 47.4% with observation; the prolonged-follow-up OS difference was not statistically significant | Integrity: V2-CONFIRMED PubMed/Crossref; no correction/retraction found
31. **S29:** Martin P et al. Treatment Outcomes and Roles of Transplantation and Maintenance Rituximab in Patients With Previously Untreated Mantle Cell Lymphoma: Results From Large Real-World Cohorts. *J Clin Oncol.* 2023. PMID 35763708; DOI 10.1200/JCO.21.02698 | Design/population: Retrospective real-world cohorts; Flatiron N=4,216 plus independent validation N=1,168 | Verified extraction: Maintenance rituximab after BR was associated with longer real-world time to next treatment and OS than BR alone; observational design does not prove causality | Integrity: V2-CONFIRMED PubMed/Crossref; comment linked; no correction/retraction found
32. **S30:** Tisi MC et al. Long-term follow-up of rituximab plus bendamustine and cytarabine in older patients with newly diagnosed MCL. *Blood Adv.* 2023. PMID 37171620; DOI 10.1182/bloodadvances.2023009744 | Design/population: Single-arm phase II follow-up; N=57 previously untreated older patients | Verified extraction: R-BAC produced CR 91%, 7-year PFS 55% and OS 63%; no maintenance was given | Integrity: V2-CONFIRMED PubMed/Crossref; no correction/retraction found
33. **S31:** Robak T et al. Bortezomib-based therapy for newly diagnosed mantle-cell lymphoma. *N Engl J Med.* 2015. PMID 25738670; DOI 10.1056/NEJMoa1412096 | Design/population: Randomised phase III; N=487 transplant-ineligible or not considered for transplantation | Verified extraction: VR-CAP improved independently assessed median PFS to 24.7 versus 14.4 months compared with R-CHOP, with more neutropenia and thrombocytopenia | Integrity: V2-CONFIRMED PubMed/Crossref; comments linked; no correction/retraction found

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